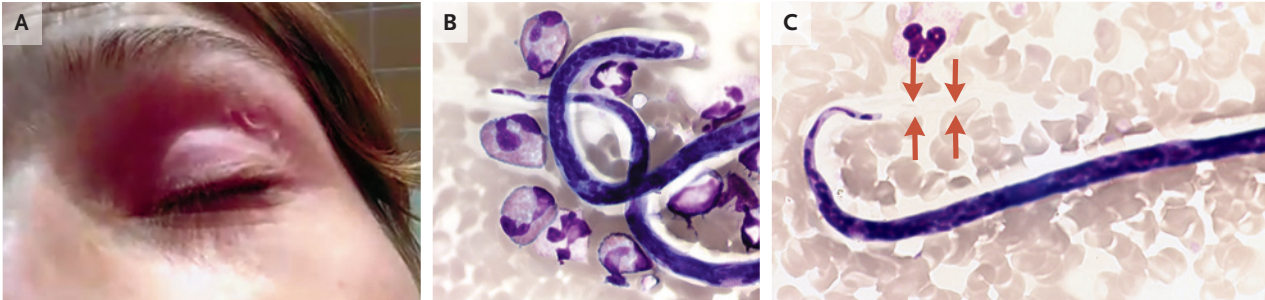


IMAGES IN CLINICAL MEDICINE

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Loiasis



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A 36-YEAR-OLD WOMAN PRESENTED TO THE OUTPATIENT CLINIC WITH A moving wormlike structure in the left eyelid (Panel A and Video 1). She worked as an anthropologist and had traveled to the Central African Republic as part of her work. Laboratory studies were notable for an absolute blood eosinophil count of 1.75 per cubic millimeter (reference range, 0.01 to 0.59). Microscopic examination of a fresh-blood wet smear revealed fast-moving worms present at a maximum count of 3400 per milliliter (Video 2). Giemsa-stained blood films confirmed the presence of microfilaria (Panel B) with colorless sheaths (Panel C, arrows), a finding consistent with *Loa loa*. A diagnosis of loiasis was made. *L. loa* is found in Central Africa and is transmitted by flies of the genus chrysops. The worms migrate through subcutaneous tissues and can produce areas of inflammatory swelling (referred to as Calabar swelling). A 20-day course of treatment with oral diethylcarbamazine was initiated, starting at a dose of 50 mg daily and increasing to a dose of 300 mg daily. Prednisolone was coadministered at a starting dose of 20 mg daily and was tapered slowly over 20 days. The patient had no complications from treatment. At a 6-month follow-up visit, her eosinophil count had returned to the normal range and there was no evidence of relapse.

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